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Name: عزة سيد محمود بغدادى National ID: 27703272402623	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

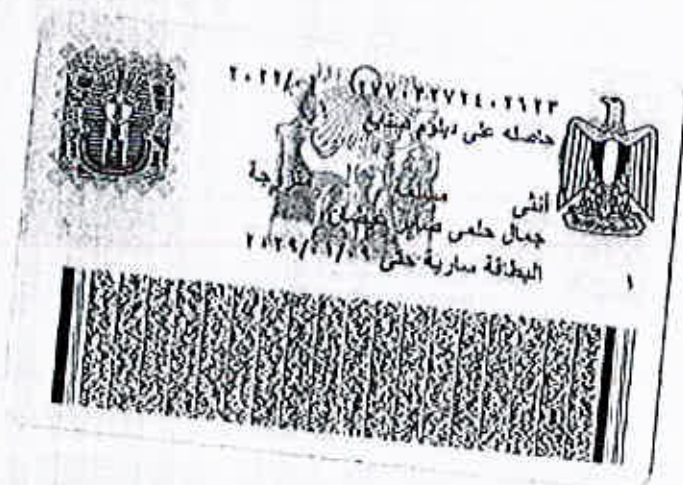
نتشرف بان نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





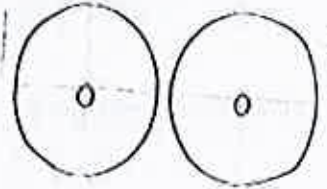
إدارة رئيس الجمهورية
للمرأة المصرية

قيد
11/11

٢٠٧٧
٢٠٢١

٢٠٧٧/١١/١١ MDT FORM

Name	سيدة - ١٢
Age	٤٤
MRN	
ID	٢٧٧٠٢٧٨٢٩
Co morbidity	١١/٩٩٠٢٧٧



Clinical Data

Dx:	IUG	SM:	
IHC:		Mets Workup:	
ER:	+		
PR:	+		
Her2:	-		
Ki67:	50%		

T N M
2 0 0

COMMENT:

Lt WLE + SLNB

Known Case of Lt breast Cancer, underwent Surgery 2 times, the first one with inadequate no pathology, then assessment revealed residual. So, underwent the 2nd surgery with WLE + SLNB. Lt assessment showed mass 2cm, residual after 2nd surgery. 3x3 So

MDT OUTCOME & PLAN

Send for 1st So for 2nd monthly T2, 2 months with negative nodes for

- ① 4 Cycles TC followed by
- ② Post operative radiotherapy followed by
- ③ Hormonal therapy TAM + 2 cycles X → shift to Femara, Zolodex

MEDICAL ONCOLOGY	SIGNATURE	SURGERY	RADIOLOGY	PATHOLOGY

Name: MR. :	عزه سيد محمود 4555	
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient was diagnosed in September 2021 with left breast cancer and underwent breast-conserving surgery (LT BCS) with axillary lymph node dissection (ALND). Pathology was staged T1 N0/7 M0. Adjuvant tamoxifen therapy was initiated in January 2022, and an aromatase inhibitor (AI) was added in June 2024. In February 2026 the treatment record was updated to reflect the same surgical and pathological details (LT BCS + ALND, T1 N0/7 M0) with ongoing tamoxifen since January 2022 and AI since June 2024. The note also indicates "ADJ TC X 4" and the addition of Zometa for Femara, with Zometa listed separately. In April 2026 the patient received Zoladex on the day of the visit for Femara therapy, noting that Ministry of Health approval allowed the use of Zoladex and Femara in place of tamoxifen, with a plan to later initiate tamoxifen. In August 2026 the note records that the patient is "sm 5/2026 free for Zometa and Femara," and that Zoladex is scheduled with a due date of 21 September. The current plan is to continue Zometa and Femara as indicated, with Zoladex pending administration on the specified date.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
	الطبيب المعالج : نهى سعيد علي محمد	
يعتمد ,		



Chest:

- * No FDG avid lung nodules or masses detected.
- * No metabolically active pericardial or pleural lesions detected.

Abdomen and Pelvis:

- * Physiologic diffuse FDG uptake is seen over average sized liver with homogenous parenchymatous texture. No FDG avid focal lesions detected.
- * Normal appearance of the spleen, kidneys, pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.

Musculoskeletal:

- * Normal FDG activity is seen over the axial skeleton. No focal active blastic or lytic bone lesions detected.

Conclusion:

PET-CT for a known patient with managed left breast cancer shows:

- > Negative study for FDG avid residual neoplastic disease or active metastatic deposits.

Much Obligated;
Dr. Aya Ashraf, MD
Nuclear medicine

Name: <i>Amo Hoyer</i>
Date: <i>11/20/21</i>

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: ..

**** referred case of breast cancer.**

1. Chest

- Few bilateral scattered pulmonary peripheral and pleural-based tiny nodular densities (2-4 mm)
- Otherwise, no pulmonary masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is mildly enlarged with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- Left adnexal cyst is seen measuring 3.3 x 3 cm.
- No abdominal or pelvic lymphadenopathy or collections.
- No destructive lytic or sclerotic osseous lesions.

OPINION:

- Few bilateral scattered pulmonary peripheral and pleural-based tiny nodules, of benign nature, for follow up.
- No hepatic focal lesions.

Dr. M. Gomaa, M.D.

Name	Azza Sayed	Date	10/8/2021
Ref. By	Prof. Dr. Essam Yhya		

Real Time Right Breast Sonography**FINDINGS**

- Normal skin and subcutaneous tissue.
- Left breast shows normal parenchymal pattern with mixed fibro-glandular and fatty elements.
- Left breast 2 o'clock well defined hypoechoic irregular soft tissue lesion is seen measuring about 3x2.2x3.2cm.
- No duct ectasia.
- Suspicious axillary lymph nodes are noted with thickened cortex and eccentric sinus fat measuring about 2cm.

OPINION

- Left breast 2 o'clock BIRADS IVc lesion with suspicious lymphadenopathies, for biopsy.

Dr., Ahmed Shawky, MD

- BI-RADS 0: Incomplete
 - need additional imaging evaluation (additional mammographic views or ultrasound) and/or for mammography, obtaining previous images not available at the time of reading
- BI-RADS 1: Negative
 - symmetrical and no masses, architectural distortion, or suspicious calcifications
- BI-RADS 2: Benign
 - 0% probability of malignancy
- BI-RADS 3: Probably benign
 - <2% probability of malignancy
 - short interval follow-up suggested
- BI-RADS 4: Suspicious for malignancy
 - 2-94% probability of malignancy
 - for mammography and ultrasound, these can be further divided:
 - BI-RADS 4A: low suspicion for malignancy (2-9%)
 - BI-RADS 4B: moderate suspicion for malignancy (10-49%)
 - BI-RADS 4C: high suspicion for malignancy (50-94%)
 - biopsy should be considered
- BI-RADS 5: Highly suggestive of malignancy
 - >95% probability of malignancy
 - appropriate action should be taken
- BI-RADS 6: Known biopsy-proven malignancy



شركة



DMS
لک سوار علی الشی والایط
لک سوار علی البیوت والموخن
لک سوار



Patient's Name:

Azza Said Mahmoud Baghdadi

Date:

2026-05-06

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Σ 000

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after left conservative surgery.

- **Breast density:** Scattered densities of glandular tissue ... *Matching ACR B.*
- **Focal asymmetry is seen in the left upper outer quadrant .**
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in both breasts.
- Normal skin and nipple on both breasts.

By complementary bilateral breast ultrasound:

- Homogenous (predominantly fatty) parenchymal echo-texture of both breasts.
- Bilateral tiny cysts are seen.
- **Clear operative bed on the left upper outer quadrant.**
- Normal skin and nipple on both sides...
- No detected significant axillary lymph nodes.

CONCLUSION:

- **Post conservative surgery changes of the left breast (BIRADS 2 Category).**
- **Right breast :Normal (BIRADS 1 Category).**

MUCH OBLIGED

Prof.DR.MONA ZAKY

Patient Name Azza Sayed Mahmoud	Date 6 / 05 / 2026
Exam : Ultrasound ABDOMEN & PELVIS	

ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:

The liver

Is enlarged in size measures 18cm at Right MCL and showing homogenous echo pattern.

No definite focal lesion is seen. No intrahepatic biliary dilatation is noted.
The Portal vein is patent.

The gallbladder No Stones or mud

The common bile duct is normal in caliber.

The pancreas Masked by gases.

The spleen is average in size, homogenous echo pattern, No focal lesions.

Both kidneys are normal in size and shape and echogenicity, normal parenchymal thickness and CM difference.

No stones or back pressure changes on both sides.
No dilated pelvicalyceal system,

The urinary bladder is adequately filled, no mass or stones.

Uterus and adnexa

No adnexal mass or free fluid is seen.
No ascites is noted.

OPINION:

hepatomegaly for lab correlation

Best regards

Dr. Mahmoud Ashour

Patient Name: Azza Sayed Mahmoud Baghdadi	Date : 25/08/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 48 Year(s)
Medical ID: 3566/2021	Pat. ID: 18439

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after left conservative surgery.

- Breast density: Predominant fatty tissue ... *Matching ACR A.*
- Focal asymmetry is seen in the left UOQ.
- No detected definite suspicious masses, architectural distortion or pathological calcifications in the both breasts.
- Normal skin and nipple in the right breast.

By complementary bilateral breast ultrasound:

- Heterogeneous (predominantly fatty) parenchymal echo-texture.
- Clear left operative bed, no detected tumor residue or recurrence.
- No detected suspicious solid or cystic masses in both breasts.
- No detected significant axillary lymph nodes.

CONCLUSION:

- Post conservative surgery changes of the left breast (BIRADS 2 Category).
- Normal study of the right breast (BIRADS 1 Category).

MUCH OBLIGED

DR. MONA ZAKY, MD.

Prof-Breast imaging-Mansoura University

Signature Dr. Mona Zaki
25/08/2025 12:16 PM



EA403674895.9

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ج. حرم، سابقا
إدارة الجودة

تذكرة العيادة الخارجية

[illegible]

General Examination

مراجعة

Vital signs:

ضبط الدم	درجة الحرارة	التقيض	الطول	الوزن
				توقيع التمريض

Marital History:	☐ Married	☐ Single	☐ Widow	☐ Divorced
Menopausal Status:	☐ Menopausal	☐ Pre-menopausal	☐ Postmenopausal	
Children:	☐ Yes	☐ No	Number:	
Allergy:	☐ Yes	☐ No	Comment:	
Adverse drug reaction:	☐ Yes	☐ No	Comment:	
Smoking:	☐ Yes	☐ No	Comment:	
Pain:	☐ Yes	☐ No	Comment:	

Complaints: Lt. Dyslexia 7/22 1.5 months

Lt 22 Nov excision (cut back) imp. pit = 11
not pathology.

Present History: 1921 - 1922 1923 - 1924 1925 - 1926 1927 - 1928 1929 - 1930 1931 - 1932 1933 - 1934 1935 - 1936 1937 - 1938 1939 - 1940 1941 - 1942 1943 - 1944 1945 - 1946 1947 - 1948 1949 - 1950 1951 - 1952 1953 - 1954 1955 - 1956 1957 - 1958 1959 - 1960 1961 - 1962 1963 - 1964 1965 - 1966 1967 - 1968 1969 - 1970 1971 - 1972 1973 - 1974 1975 - 1976 1977 - 1978 1979 - 1980 1981 - 1982 1983 - 1984 1985 - 1986 1987 - 1988 1989 - 1990 1991 - 1992 1993 - 1994 1995 - 1996 1997 - 1998 1999 - 2000 2001 - 2002 2003 - 2004 2005 - 2006 2007 - 2008 2009 - 2010 2011 - 2012 2013 - 2014 2015 - 2016 2017 - 2018 2019 - 2020 2021 - 2022 2023 - 2024 2025 - 2026 2027 - 2028 2029 - 2030 2031 - 2032 2033 - 2034 2035 - 2036 2037 - 2038 2039 - 2040 2041 - 2042 2043 - 2044 2045 - 2046 2047 - 2048 2049 - 2050 2051 - 2052 2053 - 2054 2055 - 2056 2057 - 2058 2059 - 2060 2061 - 2062 2063 - 2064 2065 - 2066 2067 - 2068 2069 - 2070 2071 - 2072 2073 - 2074 2075 - 2076 2077 - 2078 2079 - 2080 2081 - 2082 2083 - 2084 2085 - 2086 2087 - 2088 2089 - 2090 2091 - 2092 2093 - 2094 2095 - 2096 2097 - 2098 2099 - 2100 2101 - 2102 2103 - 2104 2105 - 2106 2107 - 2108 2109 - 2110 2111 - 2112 2113 - 2114 2115 - 2116 2117 - 2118 2119 - 2120 2121 - 2122 2123 - 2124 2125 - 2126 2127 - 2128 2129 - 2130 2131 - 2132 2133 - 2134 2135 - 2136 2137 - 2138 2139 - 2140 2141 - 2142 2143 - 2144 2145 - 2146 2147 - 2148 2149 - 2150 2151 - 2152 2153 - 2154 2155 - 2156 2157 - 2158 2159 - 2160 2161 - 2162 2163 - 2164 2165 - 2166 2167 - 2168 2169 - 2170 2171 - 2172 2173 - 2174 2175 - 2176 2177 - 2178 2179 - 2180 2181 - 2182 2183 - 2184 2185 - 2186 2187 - 2188 2189 - 2190 2191 - 2192 2193 - 2194 2195 - 2196 2197 - 2198 2199 - 2200 2201 - 2202 2203 - 2204 2205 - 2206 2207 - 2208 2209 - 2210 2211 - 2212 2213 - 2214 2215 - 2216 2217 - 2218 2219 - 2220 2221 - 2222 2223 - 2224 2225 - 2226 2227 - 2228 2229 - 2230 2231 - 2232 2233 - 2234 2235 - 2236 2237 - 2238 2239 - 2240 2241 - 2242 2243 - 2244 2245 - 2246 2247 - 2248 2249 - 2250 2251 - 2252 2253 - 2254 2255 - 2256 2257 - 2258 2259 - 2260 2261 - 2262 2263 - 2264 2265 - 2266 2267 - 2268 2269 - 2270 2271 - 2272 2273 - 2274 2275 - 2276 2277 - 2278 2279 - 2280 2281 - 2282 2283 - 2284 2285 - 2286 2287 - 2288 2289 - 2290 2291 - 2292 2293 - 2294 2295 - 2296 2297 - 2298 2299 - 2300 2301 - 2302 2303 - 2304 2305 - 2306 2307 - 2308 2309 - 2310 2311 - 2312 2313 - 2314 2315 - 2316 2317 - 2318 2319 - 2320 2321 - 2322 2323 - 2324 2325 - 2326 2327 - 2328 2329 - 2330 2331 - 2332 2333 - 2334 2335 - 2336 2337 - 2338 2339 - 2340 2341 - 2342 2343 - 2344 2345 - 2346 2347 - 2348 2349 - 2350 2351 - 2352 2353 - 2354 2355 - 2356 2357 - 2358 2359 - 2360 2361 - 2362 2363 - 2364 2365 - 2366 2367 - 2368 2369 - 2370 2371 - 2372 2373 - 2374 2375 - 2376 2377 - 2378 2379 - 2380 2381 - 2382 2383 - 2384 2385 - 2386 2387 - 2388 2389 - 2390 2391 - 2392 2393 - 2394 2395 - 2396 2397 - 2398 2399 - 2400 2401 - 2402 2403 - 2404 2405 - 2406 2407 - 2408 2409 - 2410 2411 - 2412 2413 - 2414 2415 - 2416 2417 - 2418 2419 - 2420 2421 - 2422 2423 - 2424 2425 - 2426 2427 - 2428 2429 - 2430 2431 - 24

$$1. Bx \rightarrow LDC \text{ (L)} \\ \rightarrow ER \oplus / PR \oplus, H_{\text{err}} \ominus$$

Past History (hospital admission & surgery):

$$C \rightarrow \text{free}$$

Family History: _____

Medically free

10. *Coniochaeta* B.S.P. ex C. G. P. 11

1. Singly linked list mid exam



**مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية**

الطبعة المصرية
طبعة
مصر

- 1151 - 00-9

MDT FORM

Name	كزينة محمد
Age	٤٢ سنة
MRN	٢٠٦٦١٢-٠١
ID	CVV.YCV&ECTCY
Co morbidity	Medically free

Pl: internal Pl: breast
D: 11/18/2021



Clinical Data

Bx: <u>IDC GII</u>	SM: <u>NOG malignant</u> <u>locking mass</u> <u>PERADU</u>
IHC: ER: <u>+</u> 2/2 PR: <u>+</u> 4/8 HerT: <u>-</u> 0 Ki67: <u>5%</u>	<u>Mets Workup:</u> <u>CTC, tx → free</u>
T N M 2 0 0	
<u>COMMENT:</u> ① Surgical report (Pt. underwent 7 In biopsies ② mass exc. at 10/1/18	

NIDT OUTCOME & PLAN

Tr. will be for wide local area (fringe of range assessment) + SWB.

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
			



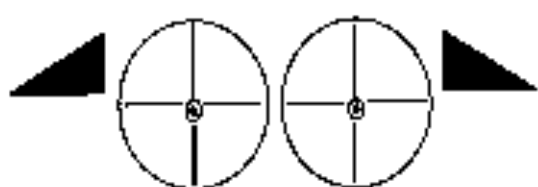
مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الاستشارة
صحة
المرأة

1077
C-C

0.0.0/1/12 MDT FORM

Name	علاء محمد
Age	44
MRN	
ID	CVV-PCDC814
Co morbidity	111990711



Clinical Data

Bx:	504	SM:	
IHC:		Mets Workup:	
ER: +ve	8/8		
PR: +ve	4/8		
Hert:	(0)		
KiTV:	50%		

T N M
2 0 0

COMMENT:

Lt wLE + SLNB
Known case of Lt breast cancer, underwent surgery 2 times, the first one with inadequate surgery & no pathology, then assessment revealed residual disease. S. underwent the 2nd surgery with wLE + SLN. Lt axillary nodes were 2 cm, residual after 2nd surgery. 3x3 so

MDT OUTCOME & PLAN
Sentinel L.N. so for breast luminal B with negative nodes.
① 4 Cycles TC followed by
② Post operative radiotherapy followed by
③ Hormonal therapy TAM + Zoladex → shift to Tamoxifen, Zoladex

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

Request No. : 136068
Request Date : 23/09/2021
Verification Date : 28/09/2021
Financial Category : توريتم المريض - نفقة التولية
Department : Out Patient
Scientific Disc. :

Request No. : 136068
Request Date : 23/09/2021
Verification Date : 28/09/2021
Financial Category : توريتم المريض - نفقة التولية
Department : Out Patient

Pathology Report

Specimen type

breast, Invasive duct carcinoma.

Specimen description

breast, WLE & Sentinel lymph nodes

Specimen Examination

WLE - 600g - 600g were received

WLE - 600g - 600g specimen: A horizontally tissue piece 7x6cm, covering skin piece 5x4cm

WLE - 600g - 600g specimen: On 5cm long. Cut section revealed a well-defined

nodules, well circumscribed, locally indurated tiny nodules (0.5x0.3cm, 0.2cm away from medial margin).

WLE - 600g - 600g specimen: A horizontally tissue piece 6x6cm containing 7 lymph nodes.

Specimen Examination

WLE - 600g - 600g specimen: Locally indurated nodules with extensive lymphocytic infiltration

WLE - 600g - 600g

WLE - 600g - 600g

WLE - 600g - 600g

WLE - 600g - 600g

WLE - 600g - 600g

WLE - 600g - 600g

WLE - 600g - 600g

Specimen

WLE - 600g - 600g

WLE - 600g - 600g

WLE - 600g - 600g

WLE - 600g - 600g

WLE - 600g - 600g

Specimen

WLE - 600g - 600g

Page 1 of 1

Printing By thermal



National Women's Health Initiative
Pathology Lab
معمل باثولوجي المبادرة القومية لصحة المرأة



Pathology Report

Name: عزه سيد محمود بكاشي Age: 41years.

Clinical data: Left breast mass

Specimen type: Tru cut biopsy.

Date (Received): 8/8/2021.

Date (reported): 16/8/2021

Lab. Number: 1714/21

Hospital Number: صحة المرأة

Gross Examination:

3 greyish white tissue cores and multiple fragmented tissue cores, up to 1cm. Totally submitted

Sections prepared from paraffin blocks for immune-staining using Bench-Mark ULTRA system for ER clone (1E2), PR clone (SP1), Her-2 clone (4B5), & Anti-Ki-67 (30-9)

Microscopic examination:

- Histological type: Invasive duct carcinoma
- Grade II
- Ductal carcinoma in situ: No
- Lymph node involvement: N/A
- Resection Margins: N/A
- Examination of ER stained slide show intense nuclear staining in >70% of the tumor cells.
- Examination of PR stained slide show intense nuclear staining in 10% of the tumor cells.
- Examination of HER2 stained slide show no staining in tumor cells.
- Examination of Ki67 stained slide show intense nuclear staining in 50% of the tumor cells.

Diagnosis:

Left breast mass, Tru cut biopsy:

- Invasive duct carcinoma grade II
- Biomarkers:

ER: Strong nuclear reaction in >70% of tumour cells. Positive, score (8/8)

PR: Strong nuclear reaction in 10% of tumour cells. Positive, score (4/8)

Her-2: Negative (Score 0)

Ki67: 50%

Prof. Dr. Gamal Abd-El Aty

Dr. Abdulrahman Ibrahim

Dr. Walea Chonaim



Patient Name: Azza Sayed Mahmoud Baghdadi	Date : 17/12/2023
Exam : MAMMOGRAPHY BOTH	Age : 46 Year(s)
Medical ID: 3556/2021	Pat. ID: 18439

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had CBS in left breast

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

The breasts are almost entirely fatty (ACR A)

No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.

Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.

Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.

No right breast solid or cystic mass lesions normal skin thickness and nipple-areola complex.

Nonspecific right axillary LN, with preserved fatty hilum and uniform cortical thickness.

No left regional pathological lymph nodes.

Opinion:

Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)

Normal study of the right breast (BIRADS 1)

Yours truly

Dr.Wafaa Ali



Patient Name: Azza Sayed Mahmoud Baghdadi	Date : 05/09/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 45 Year(s)
Medical ID: 3566/2021	18435

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had CBS in left breast

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are almost entirely fatty (ACR A).
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- No right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- No left regional pathological lymph nodes.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly

Dr. Wafaa Ali

Signature Temp

05/09/2022 12:48 PM



Total DLP (mGy cm) |:



MRI EXAMINATION OF THE BRAIN

MRI TECHNIQUE:

- Sagittal T2WI
- Axial T1, T2 and FLAIR WIs
- Coronal T1WI

MRI FINDINGS:

- Normal size and configuration of the ventricular system.
- Normal size and parenchymal signal intensity pattern of the cerebral hemispheres.
- No shift of the midline structures.
- No intra or extra axial S.O.Ls.
- No extra-axial collections.
- Normal appearance of the cerebellum, brain stem and cranio-cervical junction.
- Empty sell is noted.
- Right maxillary polypoidal sinusitis.

PINION:

- **On non - contrast MRI of the brain no intra – extra axial metastatic deposits.**

Dr. Mohammed Mahjoub

Dr. Reham Attia, FEBR, MD



Exam Information:

Patient Name: 008 On

Date of birth: 23/01/2025

Arabic Name: بقدسي محمود

Patient Gender: F

Patient RIS ID: 365014

Exam Date: 23/01/2025

Patient HIS ID: 16110225001427

Written by:

Approved by:

Dr. Hoda Fathy

Dr. Hoda Fathy

Whole body PET/CT EXAMINATION

Date: (23-1-2025).

Clinical History:

- 46 years old female patient diagnosed with managed left breast cancer.
- PET/CT requested for assessment.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCI was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. value of SUV; a Semi-quantitative analysis would be done for selected ROIs and the normal threshold is < 2.5 .

Report:

Loco-regionally:

Status post left CBS showing:

- Low grade FDG avid retro-areolar post-operative sheets with $SUV_{max} \sim 1.6$.
- Yet, no abnormal FDG uptake is seen to suspect residual neoplastic disease.

Other PET-CT side finding:

Head and Neck:

- * Normal diffuse brain activity with no hypermetabolic brain lesions.
- * Normal appearance of the nasopharynx, oropharynx, salivary glands and larynx with no FDG avid focal lesions.
- * Normal size, enhancement & symmetrical appearance of thyroid gland with no FDG avid lesions detected.



Chest:

- * No FDG avid lung nodules or masses detected.
- * No metabolically active pericardial or pleural lesions detected.

Abdomen and Pelvis:

- * Physiologic diffuse FDG uptake is seen over average sized liver with homogenous parenchymatous texture. No FDG avid focal lesions detected.
- * Normal appearance of the spleen, kidneys, pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.

Musculoskeletal:

- * Normal FDG activity is seen over the axial skeleton. No focal active blastic or lytic bone lesions detected.

Conclusion:

PET-CT for a known patient with managed left breast cancer shows:

- **Negative study for FDG avid residual neoplastic disease or active metastatic deposits.**

Much Obliged;
Dr. Aya Ashraf, MD
Nuclear medicine

Patient Name: Azza Sayed Mahmoud Baghdadi	Date : 25/08/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 48 Year(s)
Medical ID: 3586/2021	Pat. ID: 18439

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after left breast conservative surgery.

- **Breast density:** Predominant fatty tissue ... *Matching ACR A.*
- **Focal asymmetry** is seen in the left UOO.
- No detected definite suspicious masses, architectural distortion or pathological calcifications in the both breasts.
- Normal skin and nipple in the right breast.

By complementary bilateral breast ultrasound:

- Heterogeneous (predominantly fatty) parenchymal echo-texture.
- Clear left operative bed, no detected tumor residue or recurrence.
- **No detected suspicious solid or cystic masses** in both breasts.
- No detected significant axillary lymph nodes.

CONCLUSION:-

- *Post conservative surgery changes of the left breast (BIRADS 2 Category).*
- *Normal study of the right breast (BIRADS 1 Category).*

MUCH OBLIGED

DR. MONA ZAKY, MD.

prof-Breast Imaging-Mansoura University

Signature Dr. Mona Zaki

25/08/2025 12:16 PM



Patient Name: Azza Sayed Mahmoud Baghdadi	Date : 17/12/2023
Exam: MAMMOGRAPHY BOTH	Age : 46 Year(s)
Medical ID: 3566/2021	Pat. ID: 18439

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had CBS in left breast

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

The breasts are almost entirely fatty (ACR A).

No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.

Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.

Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.

No right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.

Non-specific right axillary LN, with preserved fatty hilum and uniform cortical thickness.

No left regional pathological lymph nodes.

Opinion:

Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)

Normal study of the right breast (BIRADS 1)

Yours truly

Dr.Wafaa Ali



Patient Name: Azza Sayed Mahmoud Baghdadaj	Date : 05/09/2022
Exam : Ultrasound BREAST (BOTH SIDE)	Age : 45 Year(s)
Medical ID: 3556/2021	

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had CBS in left breast

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are almost entirely fatty (ACR A).
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected
- Left breast UCQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast UCQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- No right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- No left regional pathological lymph nodes.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly
Dr. Wafaa Ali

Signature Tamp
05/09/2022 12:48 PM

Presidential Program for Women's Health

Name: azza sayed mahmoud

Date: 7/08/2021

Bilateral Full field Digital Mammography

Clinical summary:

Technique

Mediolateral oblique and CC views

High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibroglandular densities (ACR category 2)
- Left UOQ irregular dense mass lesion with no enhancing features and no microcalcifications
- Ultrasound demonstrated
- Irregular hypoechoic mass lesion measuring 26x22mm at 12 o'clock
- Normal skin/nipple/areolar complex.
- Non specific axillary LNs measuring 15-3mm with preserved architecture

Right breast

- Scattered fibroglandular densities (ACR 2)
- No masses, asymmetric densities or architectural distortion or microcalcifications.
- Ultrasound demonstrated
- No solid or cystic lesions
- Normal skin/nipple/areolar complex
- Preserved axillary LNs.

Conclusion:

Left breast: UOQ malignant looking mass for histopathological correlation will be done today (BIRADS 4c).

Right breast: Normal study (BIRADS I).

Dr. Eng. Abdel
Ghased Imaging Consultant
in Breast Intervention



Patient Name: Azza Sayed Mahmoud Baghdadi	Date : 05/09/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 45 Year(s)
Medical ID: 3566/2021	

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had CRS in left breast

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are almost entirely fatty (ACR A).
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabecularities and skin thickness.
- Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- No right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- No left regional pathological lymph nodes.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CRS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly

Dr.Wafaa Ali

Signature Temp

05/09/2022 12:48 PM

Patient Name: Azza Sayed Mahmoud Baghdadi	Date : 05/09/2022
Exam : ultrasound ABDOMEN& PELVIS	Age : 45 Year(s)
Medical ID:	Pat. ID: 18439

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- average in size (16cm at Rt MCL) homogenous parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average size , homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion :

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected

Opinion:

Un remarkable study

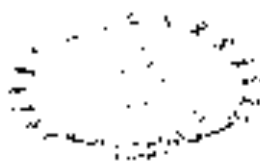
Signature Dr . Nermien Ramadan
05/09/2022 10:38 AM

Patient Name: Azza Sayed Mahmoud Baghdadi	Date : 05/09/2022
Exam : X-RAY CHEST	Age : 45 Year(s)
Medical ID:	Pat. ID: 18439

Plain X-ray Chest (PA view) revealed:

- *Clear both lung fields.*
- *Clear both costo-phrenic angles.*
- *Normal cardiac size & configuration.*
- *Normal thoracic cage*

Signature Dr . Nermien Ramadan
05/09/2022 10:38 AM



Name: Azza Sayed

Date: 1-5-2021

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

**** referred case of breast cancer.**

1. Chest:

- Few bilateral scattered pulmonary peripheral and pleural-based tiny nodular densities (2-4 mm)
- Otherwise, no pulmonary masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is mildly enlarged with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- Left adnexal cyst is seen measuring 3.3 x 3 cm.
- No abdominal or pelvic lymphadenopathy or collections.
- No destructive lytic or sclerotic osseous lesions.

OPINION:

- ***Few bilateral scattered pulmonary peripheral and pleural-based tiny nodules, of benign nature, for follow up.***
- ***No hepatic focal lesions.***

Dr. M. Gomaa, M.D.



Nile Lab
Clinic Laboratory

دائماً معك

معامل
نيل لاب
للتحاليل الطبية

Patient Name	السيدة/ عزيزة سيد محمود	Patient ID.	024051990088
Age / Sex	46 Years / Female	Request Date	19-May-2024 06:11 PM
Referred By	د. / مستشفى اورام	Printed In	21-May-2024 07:27 PM

ENDOCRINOLOGY REPORT

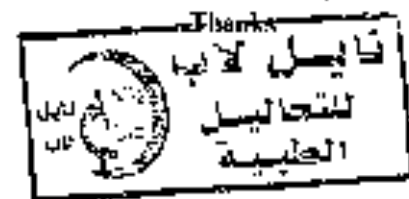
Fertility Hormones:

Reference Range :

▲ E2 (Estradiol) : 7.6 pg/mL

Follicular : 12.5 - 166
Ovulation : 85.8 - 498
Luteal : 43.8 - 211
Postmenopausal : 5 - 54.7

Doctor's Signature:



تسليم النتائج من خلال واتس أب

الهواتف (١) : آخر كويري المهران - أمام مكتب الصحة - برج البحري - الدور الثالث ت : 01149898289 العمل منوطاً خلال اليوم (٢٤) خدمة ٢٤ ساعة

الهاتف (٢) : أول تار فيلوريس - د. منور عبد المصطفى - المرقا
الهاتف (٣) : فرع مصر الجديدة - برج مصر الجديدة - المرقا
الهاتف (٤) : فرع مصر الجديدة - برج مصر الجديدة - المرقا
الهاتف (٥) : فرع مصر الجديدة - برج مصر الجديدة - المرقا
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الهاتف (١٠) : فرع مصر الجديدة - برج مصر الجديدة - المرقا



PERSONAL HISTORY التاريخ المرضي

Patient name: عبد السيد محمود Mr no:

Age: 42 Sex: ☐ Male ☐ Female

Occupation:

Marital History: ☐ M ☐ S ☐ W ☐ D

Children: ☐ Yes ☐ No number

Special Habits: ☐ Smoking ☐ Coffee ☐ Alcohol

Allergy: ☐ Yes ☐ No Comment:

Adverse drug reaction: ☐ Yes ☐ No Comment:

Complaints:

Present History:

PTA PNa M by CT 28/9/2021
ER = 818

PR = 418

ITC =

KIT 50%

MDT Fa adj 4 TC → RORT → HI
TAM + Zoladex

Past History (hospital admission & surgery):

• surgical:

• Diabetes mellitus:

• Blood Transfusion:

Family History:

Psychosocial History: Asymptomatic

Checked & Reviewed by Dr.

Date: Time:



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقا)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض:

الرقم الطبي:

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

gastro-intestinal healed

For labs:

+ Dexamethazone 8mg

علاج فموي من الستيرويدات

2. x 3 x 10

توقيع الطبيب:

التاريخ:

الوقت:

د. أمال محمد

Apixan tea 2 x 3 x 10

2. x 3 x 10

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

ملاحظة

2021/11/18

توقيع الطبيب:

التاريخ:

الوقت:



Wt: 96
Ht: 160
SA: 2

وزارة الصحة والسكان
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مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :

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وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي :

الأوامر

chest:

Temp:

Oral mucositis:

Canula site infection:

ملحوظة:

For CPT

توقيع الطبيب :

التاريخ: / /
الوقت:

C. 150 K 0.01

الأوامر

chest:

Temp:

Oral mucositis:

Canula site infection:

ملحوظة: 10.8 mg SC

Teramoxol 10.8 mg SC

Calcium 10.8 mg SC

One alpha 10.8 mg SC

توقيع الطبيب :

التاريخ: / /
الوقت:

في 11/10
C. 150 K 0.01
د. محمد
م. محمد



Doctor's Orders أوامر الطبيب

الأوامر

Chest: _____
Temp: _____
Oral mucositis: _____
Canula site infection: _____

تم إجراء فحص الصدر
والأذن والحنجرة
والتنفس طبيعي
القلب والرئة طبيعية
لا توجد شكاوى أخرى

توقيع الطبيب: _____
Cm/r/cg



وزارة الصحة والسكان
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مركز أورام دار السلام
(هرمل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي :

الأوامر

chest:

Temp:.....

Oral mucositis: _____

Cat-scratch disease.....

10mm x fm 2mg | X | X 9p

Zachary D. B. 0.8

Calicut, 21.12.90

Graphs of $f(x)$ and $g(x)$

Neural 1 x 1 x 9 =

Graph 400 m [x + 1.9]

التاريخ:
الوقت:

— 14 —

الأوامر

c - c { 7 / 50 }

CHART: Calvin

Temp: 77.4 F.

Oral mucositis: ☒ Yes ☐ No

Chlamydia site-infection	100
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9. 75 X 1 1/2

9. $X_{11} = 1$

9. XIX WILL

P. XYX] — — — — —

W. H. W. (C) 1964

توفيق الحبيب : د.

التاريخ:
الوقت:

۱۰۱۸
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10. 4/9/12



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أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقا)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي :

الأوامر

Chest:
Temp: 36.8 / 37.5
Oral mucositis:
Canula site infection:

Dr. DEVA

توقيع الطبيب :
التاريخ : / /
الوقت :

الأوامر

Chest:
Temp:
Oral mucositis:
Canula site infection:

توقيع الطبيب :
التاريخ : / /
الوقت :



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(هامل سابقاً)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :

الرقم الطبي :

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

Dentist

Patient is free from any septic

focus & she can tolerate Zolozon

Dr. Mohamed Shams

18/11/2024

توقيع الطبيب :

التاريخ:

الوقت:

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

أولادكم اسير تحت الحرج

بأمر زعيم راحم ربكم

في الساعة ١٢:٠٠ من يوم الاثنين

رام الله بكم محمد بن عبد الله

بأمر زعيم راحم ربكم

بأمر زعيم راحم ربكم

توقيع الطبيب :

التاريخ:

الوقت:

الوقت:

١٤١١ / ١١ / ١٨

شهور علاج هو مونا ونوس

زوجة دكتور



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقا)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض :
الرقم الطبي :

الأوامر

Chest:

Temp:

Oral mucositis:

Canula site infection:

Sr.Ca: 1.08

Sr.Cr: 0.8

دكتور ربيع 10/8/2018

صغار

تكملة

500

في حاد

جاء

توقيع الطبيب :

التاريخ: 10/8/2018

الوقت: 10:00

Co. +

الأوامر

Clinical pharmacy

Postpone

مبارك

C.Ca: 1.08

Chest:

Temp:

Oral mucositis:

Canula site infection:

أحمد

10/8/2018

10:00

Co. +

الأوامر

توقيع الطبيب :

التاريخ: 10/8/2018

الوقت: 10:00



وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مركز أورام دار السلام
(هرمل سابقا)
إدارة الجودة

Doctor's Orders أوامر الطبيب

اسم المريض : ع.ق.ن. السيد محمد م. و. و.

الرقم الطبي :

الأوامر

chest:

Temp:

Oral mucositis:

Canula site infection:

wt 90

Ht 160

Age 67y

- Zometa 4mg (After Lab.)

- Zaladex 1x8

- Fennar

- Calixan

- one alpha

- Miga

التاريخ: 28/7/2025

الوقت: 11:00

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الأوامر

chest:

Temp:

Oral mucositis:

Canula site infection:

- M Zomara

- M Glemate

- M onealpha

- M Panadol 2x3x10

التاريخ: 28/7/2025

الوقت: 11:00

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توقيع الطبيب: 28/7/2025

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